

OFFICIAL

Community Mental Health Appointments Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Health Service	Community Mental Health Services	Medical, Nursing, Clerical

2. Purpose

To provide a clear framework for mental health clinicians to manage consumer appointments within the care coordination framework [EMHS Care Coordination in Mental Health Services](#)

3. Background

To optimise consumers' engagement, manage crisis, reduce the risk of relapse and support meaningful recovery, the clinical risk issues and barriers associated with the non- attendance of consumers and the rescheduling of community outpatient appointments; a consistent approach is used across service locations in Armadale Health Service, Mental Health.

Any medical appointments rescheduled should be communicated to the consumer via Essendex, and an email sent to the relevant Care Coordinator by the clerical staff to confirm new appointment details. All medical appointment scheduling is communicated to the consumer via automated SMS messages from WebPAS, and an appointment letter can be generated and either handed personally to the consumer or sent by Australia Post for appointments more than three weeks in the future.

All medical appointments with consumers are entered to the appropriate WebPAS clinic by clerical staff. No scheduled medical clinics in all programs should be rescheduled without permission of the Program Manager or Team Leaders.

At all times in the event of a clinic being rescheduled, the Mental Health Act status of the consumer should be considered prior to determining cancellation, as some consumers have obligations under the Mental Health Act 204 (MHA) to be reviewed within a minimum time frame.

Care Coordinators have view only access to WebPSOLIS and are able to view clinic diaries to ascertain clinic changes and availability, to inform clerks regarding appointment confirmations.

3.1 Rescheduling Medical appointments all Community Teams

3.1.1 Consumer requests to reschedule an appointment

If the appointment to be rescheduled is an Initial Assessment:

- Clerical staff will refer the consumer to their Care Coordinator/Allocated Clinician; or ATT/OA Triage or CTT First on Call if the Care Coordinator/Allocated Clinician is unavailable. The clinician will review the Digital Medical Record (DMR) for any pertinent issues including risks and make a clinical determination if rescheduling should occur.

Before referencing this procedural document please ensure that you have the latest version from the [Policy Library](#) information hub page.

The consumer or personal support person will be informed of the time scale of the proposed new appointment.

- The Clinician facilitating the change of appointment will liaise with clerical staff with regards to rescheduling. This may include checking for the earliest available appointment on WebPAS with the other team doctors, unless a specific doctor has been indicated.
- The rescheduled appointment will be confirmed with the consumer via Essendex and a letter if the rescheduled appointment is more than three weeks in the future.
- If the reschedule has occurred in the absence of the Care Coordinator, an email is sent for their information by clerical staff.

3.1.2 Consumer requests to bring forward scheduled appointment

- Process as per rescheduling appointment.
- The identified clinician will speak with the consumer or personal support person to determine the clinical indication and urgency of the request against appointment availability.
- The identified clinician will note in the health record the change of appointment time and advise clerical staff, to reschedule on WebPAS. Clerical staff will advise the consumer via Essendex of the new appointment details and send a WebPAS appointment letter if the appointment is more than 3 weeks' away.
- appointment

3.1.3 Consumer requests to defer a scheduled appointment

Should a consumer (not on a Community Treatment Order (CTO)) request their scheduled appointment be changed to a later date:

- As per process outlined in 3.1.1
- The Care Coordinator/Allocated Clinician will liaise with clerical staff if the scheduled appointment requires changing, based on their clinical assessment.
- Clerical staff will proceed with the request and communicate the change to the Care Coordinator/Allocated Clinician and confirm the reschedule with the client via Essendex text message.

If the consumer is on a CTO and requests their appointment to be rescheduled:

- The request to reschedule should be referred to the Care Coordinator/Allocated Clinician; or if the consumer's Care Coordinator/Allocated Clinician is unavailable, the Triage ATT/OAMH or First on Call CTT/EEPT to discuss with the treating Consultant Psychiatrist to determine the earliest review date available.

3.1.4 Rescheduling of Medical appointments by OAC

If a doctor's whole clinic is rescheduled:

- Clerical staff will print the WebPAS clinic diary page(s) with new appointment information added and place the printout on the front reception desk.
- Clerical staff will contact all affected consumers to notify of rescheduling via Essendex text message or by telephone.
- The clerical staff will send an email to all clinic staff advising of the rescheduling, and for the staff to check the printed WebPAS clinic diary page(s).

3.1.5 Consumers who 'Did Not Attend' Medical Appointments

When a consumer DNA a scheduled medical appointment the doctor is responsible to follow-up to ensure safety of the consumer in the first instance. Clerical staff will await outcome and direction from the clinical team before rescheduling any appointment following a DNA.

- The relevant doctor will call the consumer within the timeframe of the appointment time to evaluate the welfare of the consumer and ascertain the reasons for the consumer not attending (DNA). The evaluation of the consumer's welfare should be documented in the Digital Medical Record (DMR) as per [EMHS Clinical Documentation Policy](#)
- The doctor will make an entry in DMR of the attempted phone conversation; outcome and plan of action e.g. follow up by Care Coordinator/Allocated Clinician to reschedule appointment if the consumer does not answer via the contact numbers identified in the digital medical record (DMR) and PSOLIS. They will communicate this plan directly with the clinician tasked with the outcome at the earliest opportunity.
- The doctor will attempt to contact the consumer's Personal Support Person(s), if the consumer has given prior consent, or if clinically indicated that this contact is necessary; and / or the consumer's General Practitioner (GP) if more information is required on risk and mental state.
- Should the consumer decline to speak to the doctor, the doctor should refer the call to the care coordinator/ triage or FOC to make further enquiry surrounding reason for non-attendance and an evaluation of the welfare of the patient.
- Consideration will be made by the Doctor as to whether a home visit at this time is indicated to evaluate the welfare of the consumer if there are concerns for their safety and/or wellbeing.
- If the Care Coordinator/Allocated Clinician is unavailable and the doctor believes there may be risk to the consumer or others which may require more urgent follow-up, this should be discussed with the ATT/OA Triage staff, or FOC CTT/EEPT or a request made to the ATT afterhours staff. Feedback about the action taken and risk assessment of the consumer if undertaken, is to be communicated to the Care Coordinator / Allocated Clinician / treatment team.
- When a consumer scheduled for an ATT initial assessment DNA, the care coordinator will contact the consumer to organise an initial assessment at a mutually agreed time.
- Calls should be attempted for two consecutive days following a DNA, if no response has been obtained this should be escalated at the earliest opportunity and a clear plan documented in DMR.

3.1.6 For non-attendance of all other scheduled clinic appointments including Long Acting Injectable (LAI) administration, Psychology and Care Coordination appointments

- The relevant clinician will endeavour to call the consumer within the timeframe of the appointment time to evaluate the welfare of the consumer and ascertain the reasons for the consumer not attending the appointment. The evaluation of the consumer's welfare should be documented in DMR
- An entry in the consumer's file of the attempted phone call; outcome and plan of action e.g. reschedule, follow up by Care Coordinator/Allocated Clinician and/or ATT involvement.

- If the relevant clinician is unsuccessful in speaking with the consumer, then attempts will be made to contact the consumer's Personal Support Person(s), if the consumer has given prior consent, or if clinically indicated that this contact is necessary; and / or the consumer's GP if more information is required on risk and mental state.
- Calls should be attempted for two consecutive days following a DNA, if no response has been obtained this should be escalated at the earliest opportunity with discussion with members of the multidisciplinary team for decision making and a clear plan documented.
- Discharge decisions based solely on the person DNA appointments should not be determined, without documented phone or face to face contact attempts and documented contact to the referrer.
- The ATT afterhours service will be utilised as indicated following escalation of care with the Multi-disciplinary team.

3.2 Escalation of Care

Failure to attend an appointment or being unable to contact a consumer or next of kin should be escalated to provide the most appropriate and timely intervention. Escalation of care can include but is not limited to MDT discussion at clinical review, stand up meeting, speaking with treating doctor or supervising psychiatrist.

All escalations of care should be documented in DMR and detail: date/time of discussion, clinicians present, current situation, identified by plan and the clinician responsible for the action.

If the outcome of the above actions is that a new appointment is to be made, the clinician is to liaise with clerical staff to schedule a medical appointment on WebPAS.

3.2.1 Failure to attend multiple appointments and discharge from service.

- Clerical staff will facilitate an SMS for all medical appointments.
- To increase engagement, care coordinators should consider sending an Essendex SMS message prior to the consumers scheduled appointment if likely to DNA
- Failure by a consumer to attend a planned appointment with either the care coordinator or doctor will be considered a valid DNA and recorded in DMR and PPSOLIS
- Consumers who fail to attend multiple appointments are to be reviewed by a clinician to determine their current mental state and risk. Repeated non-attendance is to be discussed via the clinical review meetings to determine if the consumer can or should be discharged for care and management to continue with a nominated GP. In a 7 day (ATT) or two week letter (CTT/EEPT) template may be sent providing the opportunity to further engage with services.
- There will be an escalation of care to the treating doctor informing them that the 7-day letter (ATT) or two-week letter (CTT/EEPT) has been sent and this will be documented in DMR
- When the 7 day or two-week deadline has passed, if no contact has occurred, the Care Coordinator will raise with the treating team who will consider all potential risk issues including potential for relapse before making the decision to transfer the care to the GP
- The decision to discharge from the service will be with the approval of the consultant psychiatrist.

3.2.2 For non-attendance of all other clinic appointments:

- The relevant clinician will endeavour to call the consumer within the timeframe of the appointment time to evaluate the welfare of the consumer and ascertain the reasons for the consumer not attending the appointment. The evaluation of the consumer's welfare will be documented in DMR and include the plan for follow up.
- If the relevant clinician is unsuccessful in speaking with the consumer, then attempts will be made to contact the consumer's Personal Support Person(s), if the consumer has given prior consent, or if clinically indicated that this contact is necessary; and / or the consumer's General Practitioner (GP) if more information is required on risk and mental state.
- Discussion with the multidisciplinary team will guide decision making.
- Discharge decisions based solely on the person DNA appointments should not be determined without documented phone or face to face contact attempts and documented contact to the referrer or other stakeholders involved in the consumer's care.
- The ATT afterhours staff will be utilised as appropriate.

4. Compliance monitoring

The Program Manager has the responsibility for monitoring rescheduled clinics and DNA rates.

5. Policy Context

The following documents support the implementation of this procedure:

- EMHS
 - [EMHS Clinical Documentation Policy](#)
- AKG
 - [Assessment Treatment Team Process Manual](#)
 - [Clinical Treatment Team Process Manual](#)

6. Supporting information

- Office of the Chief Psychiatrist
- HDWA
 - [Clinical Risk Assessment and Management \(CRAM\) in Western Australian Mental Health Services.](#)
- EMHS
 - [Care Coordination in Mental Health](#)
 - [Mental Health Consumers who Decline Assessment](#)
 - [Care Coordination in Mental Health Services](#)

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.08

National Standards for Mental Health Services (2010):

- Standard 1 – Criteria 1.1

8. Document owner

Service Director Mental Health

Enquiries relating to this procedure may be directed to:

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9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	03 02 2026	03 02 2029	Full Review, title update and significant changes throughout document

10. Authorisation

This procedure has been authorised and issued by:

Authorised by	Dene Olive – A/ Service Director Mental Health
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